

[Portion Redacted]

Interviewer: *So when your clients come to you, I know there's bound to be a broad range of issues, but what usually is the presenting issue or the reason that they seek your service.*

Therapist: Right? So as I said, most of the couples I get in, they will be fairly young, not in a terribly like, you know, long relationships. Most of the time it's like a mixed or mixed culture sort of thing. One of them typically is [Nationality], either [Nationality] resident, and then they have like an expat or somebody like a foreign spouse or partner who's coming who's relocated here. But it could also be just two foreigners living in [Country]. So a lot of that I mean, it's not necessarily like the presenting issue, but it might be like a contributing factor would be some of the cultural differences and just the fact that they're living in [City], as is a foreigner. So it would be I mean, in general it would be, as they put it, location issue. So frequent arguments there will be there will be growing distance, emotional disconnection. I had some cases of infidelity, some jealousy issues. I think that's that's the most of it really is. They would call it like, you know communication or gap something along those lines. Typically it's connected to just growing this connection to the point where they can they feel like they can speak to each other. They need help or they're they're coming in at the last resort typically. So so they would be either thinking of separation and just wanted to get some decision around that, get some clarity.

Interviewer: *Yeah. Interesting. And I know that in our email you said that you don't do a formal assessment upon an assessment.*

Therapist: Right? So yeah, I was thinking about that because, I mean, I'd say like the situation changed by me a little bit. I'm on parental leave currently since we since we had the COVID pandemic as well. I went online only so before that. So that be like [1-5] years ago when I used to meet the couples in person, I actually did. So I do like I made with a couple together. Then I do an individual session and then I continue meeting together with them. So in the individual session, the way I would handle it is just to have like a split it into half and have like an individual meeting with one partner and then hand in some assessments to the other person. So as they were waiting, they would sort of fill that in. So since I'm doing, I don't know, I just transferred online. I'm not really doing that anymore. Also for just the fact that, you know, typically it's a mess even to get the consent back. And I don't have anything built in, as you know, as as the assessment tools there would be like electronic that would say, be linked to my website. So some of that would be like that part of it. The other part of it is also just because I'm getting more trained in it now. So I think it's more that you sort of think a little different in terms of assessment. But I was looking back at my what I was using before, and that would be like a DAS and what is it, the relationship satisfaction scale. So I mean, I would hand out occasionally some of them will be good information. I'm not

currently doing any of that. I don't know. Might come back to it in the future. I haven't decided yet.

Interviewer: *So how informative did you find these skills that you use to be this right and other relationships?*

Therapist: Yeah. Yes. Yes. So. Right. Let me actually I just want to give it the correct name. There is I love Yes, that was a good one also before. So when I was getting training back in the [Country] where I worked, my my internship was in a clinic. It was like a family clinic. So part of that we would have like standard intake protocols. So there will be a more of that also back and some individual, you know, history assessments, things like that. But for the relationship part, I think I did just inherit that or took from from there a DAS s I liked also because it just gives you a quick glance at the sort of the cutoff, I think like just between the sort of the married, divorced and such. Also, it covers some of the just different areas. And that's something that I am like thinking What I'm thinking about right now is because I mean, it's sort of good to map out the areas where the conflicts lie, such as parental or finances, things like that, but also like thinking, adopting more EFT. Right now it's also like I think like doesn't necessarily mean. Like, what is the specific topic? Because sort of the dynamic occurs, you know, just a pattern plays out regardless of the topic. So so that's where I'm at right now. I still think it's like it's useful. I think that in general it's the more information you have, it is, the better it's better or so. I mean, it always adds to the clinical value, but as I said, like the way I would typically handle them before if I had something, if I did collect something, it just would be nice. First of all, to contrast and look at the different perspective of the of the partners and just see if those are like, what is the difference there? If they sort of both are somewhere like, you know, disagreeing or somewhere like unhappy or both of them are, or if that's a difference. So they'll be the first thing I'd look at. Then also the cutoffs, at least for the DAS, is just to see where they're placed, how how serious are we speaking? And then sort of like take note of what is the major things, especially things around our, you know, intimacy regression, like some of the questions on that actually do like so I do like in general, I think it's informative to work with that. And again, as I said, the more information you sort of have from the beginning, the better. My my also experience is that typically when things are important, they come out, you know, during the process of therapy regardless. So it's and also like some couples or some some my experience also is that with with the clients, some sort of like that type of thing, I just find it just bothersome or I had people skip question as well if they didn't want to. So I never like that as a pressure. I always invited them to answer to what they wanted. So they're just I'm rambling here, so feel free to direct me if you have more specific questions.

Interviewer: *Rather I'm familiar, of course, for these, but with the relationship satisfaction scale.*

Therapist: Oh, I didn't. So it's just and I wonder if that's something coming from our clinic

back in the days Couple Satisfaction checklist It's 2003 [inaudible], I'm not sure if it's going to open up if I just send it to you or I can share a screen if you want to see it.

Interviewer: *If, if you're sending something by chat.*

Therapist: Yeah, let me see. I'm not sure if that's going to pop up because.

Interviewer: *Oh, perfect. There you go.*

Therapist: Yeah. I mean, it's just a simple list of. Yeah. And that, again, like not necessarily said on this one or I think that it's just something that I used to have in my back that we were using as part of our intake protocol. And I just adopted it back in the day.

Interviewer: *So did you have any challenges when you were using these two scales?*

Therapist: Well, again, as I said, like it depends on the people. Some are like you handed in early in the year, meaning that people some might be more like, know, why would I or maybe not necessarily wanting to share so much detail. So I would have like people skip questions. I think that's, you know so from the scoring perspective and don't necessarily have the full picture added and that not really some would be hesitant to. I mean I don't think I had that situation though because I would collect them individually. You know, I think now that I'm thinking about I'm not sure I had a situation of like when I handed them out like, like online because I would always try to protect the privacy and not have the other person sort of like see that. I don't think that that has happened, but I can't recall otherwise. No.

Interviewer: *And just to clarify, you then pay for the skills, right?*

Therapist: So, again.

Interviewer: *You didn't pay for these skills. These were free.*

Therapist: No, no, no, I didn't.

Interviewer: *All right. I touched on this on my email as well, that the years of the years, different therapists and different researchers characterize the construct of relationship*

quality quite differently from each other. I wanted to get your view about it for you. What is the relationship quality?

Therapist: Right, Right. That's a nice one. Yeah. I guess like because I'm sort of like trying to like am heavily like, you know, getting trained in ETF right now or trying to understand some of the concepts. So the first thing that would come up for me would be around like attachment issues or, you know, attachment theory, clearly so and obviously there are different there are different theories. But I would just I guess like from my experience, I would think about just the just emotional bonds, something where, you know, how how the partners are connecting and how like subjectively satisfied already with that, you know, how that would be fulfilling for them. And so which obviously, like fluctuates in time and could be different from from partner to partner. But I guess like to me, if I was to look at like the the quality of a partner, it would be like how they subjectively experience their sort of like relational. I don't have the good words for it. Well, being in it, you know how they can share with each other things that are connected to their vulnerabilities, to their pain and stuff like that, how they can relate to each other. So not just cope individually. That's one thing that comes up a lot is this almost like mutual dependency rather than individual coping. So yeah. Something like that.

Interviewer: *Interesting. All right. Maybe to to to tackle it from from a similar place as well. For you, for example, what differentiates couples with a high relationship quality and couples with a low relationship quality?*

Therapist: Yeah. I want to say like something along the distance, sort of like and it would have to do with like flexibility and rigidity in like if they can, because it just feels like with those that are really like on the brink of like separations, it just it's it's really threatening for them to, to do anything like mutual in a way. Like there is a lot of like there's a lines of like negative expectation that the other person is out there to hurt them really. So so that's that's to me it's like when I see really like a couple that's like that comes in and they're like, they just snap. Like it's every eye roll. It feels like they just take things personally and it's just very rigid in terms of how reactive they also, the reactivity just spikes like in the moment and moment, regardless of what you're speaking, very the opposite would be where just feels like there is a lot of, you know, like there's more flow, more flexibility. They can sort of adapt their reactions if they get reactive, which they do, it's much easier to just de-escalate that. They can also reconnect afterwards. They can speak about that. They can sort of like even themselves sort of debrief and reconnect afterwards if there wasn't a disconnect happening.

Interviewer: *So interesting. All right. So for you, I mean, of course, you mentioned these things like about their subjective well-being, about these expectations, this rigidity, flexibility. But I just wanted to maybe also focus on a particular question. For example, in your professional experience, what are the most important things in a high quality relationship?*

Therapist: So, I mean, trust comes in as far as thing almost like trust that the other person would like sort of be there if I needed them. So it's almost like it's this fundamental, like, if I need you, you'll be there. Like you have my back, right? Like, it's like this feeling that you're not the enemy, right? I mean, we are in this, I guess, like with that comes this togetherness, something like a shared sort of. It's not just me and you in the relationship, but like, we are creating this together. We share like, some vision for our lives, things like that. So I said, trust, togetherness. And then, I mean, typically to me it would have to do with like connection in terms of like physical intimacy as well. That would work as well where they can, you know, they can connect, like physically be intimate and also like, you know, like verbally obviously share like internal worlds, things like that. So just some some of that.

Interviewer: *All right. So when we were talking about your conceptualization of of relationship and relationship quality, I wanted to know if this often differ from how your clients conceptualize relationships and this difference impact the therapy. Does this matter?*

Therapist: Yeah. So it's interesting that you bring it up because I think it's not necessarily like now that I think about it, it's like not necessarily that it would like differ dramatically in terms of what what is the good relationship. I think that what differs quite a bit is just based on the like personal background and the family histories, because for some there would be and I do think sort of in like withdraw pursue pursuer positions right now or sort of they're like family of background typically, you know partners come in and they come from like highly emotional family where they're all over them, you know, themselves. They were like expressing things. And so that would sort of be their feel like that's the baseline. And then you have to especially like connected to culture, I think where people are a little more like, you know, not necessarily sharing so much, a little more like I don't want to take hold, but, you know, so I feel like the expectation would be a little different. And it's like also maybe there is a bit of a gender aspect. I'm not sure here. Also, like in terms of how it's going to be expressed, like there's there's clients who think there's a big assumption that, yes, we are together, I love you all of that. It never gets verbalized. So and they would sort of do things, acts of, you know, they would take care of things. And that's their way of expressing. And then there are others who sort of are expecting that it's going to be like verbalized all the time. So I think that there are some clashes definitely between the partners and how they perceive it and what sort of their ultimate is. And then also like for us then to sort of work together because sort of the, you know, the goal of it would be to get them to to speak to each other from a position of vulnerability. So that might be different. And I think it's it does come at odds sometimes with some of those, like more with those kinds of come from more of the withdrawn background sort of and the way we would work around that. I mean, you're just, you know, I don't know, like they're working towards melting the ice is lowering the protections. So now I'm thinking if we are like, how much of that is just like like the work itself not necessarily being like super clear and transparent about that? I'm not sure. Yeah, I'm not sure. Am I answering your question? Yeah, I mean, if I'm making sense or not. Yeah.

Interviewer: *Yeah, yeah, thank you so much.*

Therapist: Yeah. Because, like, rarely do I have a client who comes in and they give me a list of qualities and they're like, This is what we need to work on, right? So that typically doesn't happen. So yeah.

Interviewer: *I wanted to know as well because the relationship, right? It's a complex system. It involves love, sex, satisfaction, all of that.*

Therapist: Sorry, I'll just let me just pause because I have kids running around here. Oh, sorry for the disruption. Go ahead.

Interviewer: *No problem. It sounds like a happy home.*

Therapist: Yeah. Loved one.

Interviewer: *About therapy. How do you decide which one to focus on in this complex system? Which one? How do you decide which in this complex system to target?*

Therapist: And which one of which.

Interviewer: *Sorry. Because the relationship, right. Is our system of love. Identity.*

Therapist: Yeah, yeah, yeah. I mean, like, my understanding is, like, we are like, we are going for, like, connection, you know? And that's obviously a broad construct, right? But we are coming. We're going into large sort of like in the long term for like them feeling connected to each other. And so like, typically when they come in, they would be always like, Oh, we fight a lot. And there is like arguments and we feel this and that. And so I think that we are like. To me, it's I don't know, I'm sorry, but I'm like heavily thinking of 50. So to me, like, immediately what comes up would be like, we need to de-escalate conflict first in order to be able to work on work on things that are like to me, I guess like my my thinking would be that there are some stages, right? Like we don't necessarily go in in the first session and speak about love and they haven't even like, looked at each other for months. So there is definitely like things that we sort of take care of first and something some, some ideas that I

hold in my mind, like, you know, as we progress through the process. So, so I guess like it might be different also like depending on where we are at in therapy.

Interviewer: *Yeah. All right. So how do you determine when to end the couples therapy?*

Therapist: That's a good one. I think. Did we ever end it on my, you know, like, typically, like things happen. And so I feel like also part of that, like a good chunk of of couples would actually just come up to a conclusion. They are no longer a fit for each other or one person might be out, things like that. Then also I had some like breaks in my my sort of like my career with couples because of the maternity and things. So there would be some cases where it just I couldn't go on with them for the entire sort of journey. But I'm thinking of couples that would sort of like I mean, it would always be like collaborative, it would never be me, sort of me suggesting I had couples who sort of like they came in engaged in the last session. They came in and they told me they got married. And sort of like that was they did was a beautiful sort of ending and like, it was natural, right, somehow. So I guess like in general, it would be a collaborative process. We would feel like we are hitting some marks where they're reporting back progress session to session. It feels like the tension isn't so high. They're able to take care of themselves. Also, to me, a big marker is like when they sort of start not necessarily canceling session, but it feels like there is other things outside the world that's sort of more important. So we start spacing out. So typically it doesn't like just end from week to week. We sort of space out. And then also we like, we review goals like initially, you know, there's a lot that back and forth in terms of checking how they're satisfied, how they're doing. So it's a lot that I rely on what they're reporting back in terms of are we meeting the goals that we set at the beginning?

Interviewer: *Interesting. And usually for these couples, especially for those who stay together and are successful, this. Usually how long do these take?*

Therapist: Sorry, I didn't get the question. How long does what.

Interviewer: *From the start of the therapy to the termination successful terminate?*

Therapist: It's going to vary. I'd say. And again, like I've been like in my private practice and with breaks for like [1-5] years. So I want to say like. My longest couples I've seen was probably to something over [1-5] years, not necessarily longer. So and again, like I want to point out, it's also related to just a couple of today's see, like maybe because my profile and my face, like I don't typically get like married couples who've been together for like [65-70] years and that's not impossible. But you know what I'm saying? It's like where the problems are really rigid. It feels like I'm getting a lot of like couples that have been together for a couple of years. They're trying and it either works out or not. So typically you don't need,

like, you know, years and years in therapy. It's sort of like you get a sense of if this is working or not, like relatively short shortly. So yeah, I'd say [1 -5] years maybe. Yeah.

Interviewer: *And for the couples who eventually, you know, feel that they're not fit for each other, usually, how long does it take for them to.*

Therapist: Get to that point again. So different. But I had couples who just like emailed me sort of out of the blue, like they would just like let me know after a couple of sessions and it felt like we are doing progress to something happened arguments they got invite and they just let me know There were some that we've been doing like meeting for, I don't know, 20 sessions, something like that. It feels like we would work for, I don't know, half a year we would work for some time. And they just felt like it wasn't making so much difference, maybe as they hoped or also circumstances came in and so it will be different. It's really hard for me to give a general answer here.

Interviewer: *No problem. All right. So your sessions are every week. Every other.*

Therapist: Yeah. So I try to do weekly sessions, even have a couple right now who is asking for it like twice per week, which is really unusual. So typically would be a week to two weeks. I had a bunch of couples, actually, because it's it might be hard to just scheduling wise so I do typically do twice a week not sorry one time per two weeks sort of so yeah, biweekly. There you go. Biweekly is the word.

Interviewer: *All right. So either once a week or twice a week*

Therapist: Yeah, but how long it's going to be different. So again, like, are. You like length? Like one hour.

Therapist: Oh, sorry. Yeah. So I do have like I do 50 or 80 minutes and I typically like, I like my condition that for the intake session we do 80 minutes and from there it's more or less like couples choice. I have some that 50 minutes just works fine. I typically do 80 minutes because it feels like it's just too much and we can really fit in, especially if we are meeting biweekly. Then we do that. But I had couples that I saw 50 minutes as well.

Interviewer: *All right. So I you know as well from the email, we are currently developing a relationship Quality Scale that hopefully better represent the relationship as a complex system that is aimed to help therapists both in treatment, planning and treatment evaluation. How interested would you be in this deal?*

Therapist: I mean, again, I said like there is know, I feel like it's like if it's a good measure, a good tool, it always brings information. And even if it's not such a good tool, I feel like every scale that or every assessment is a piece of information that I fit in. I don't necessarily like rely all of my everything. Like I still get, I think like much more information for individual interviews, couples interviews, but definitely something that I find useful. Otherwise, I don't think I'd even be talking to you. So.

Interviewer: *All right. So what would you be looking for personally? What would you be looking for in a newly developed relationship like this? Like, how can we make sure that this scale is useful for your practice?*

Therapist: Yeah. So that's I mean, that's a good one. So definitely something that's like practical in terms of like, you know, you don't want to spend like time scoring sheets. So definitely good like evaluation scoring thing. That's just something that can be as brief as possible. And then I do really like the cutoffs. So something that just like immediately gives you it could be like subscales that's typically useful when you have like different, like, you know, different what's the word construct, right? And so but it gives you a sense of even just like, where are we at? Like is this like distressed couple or is it not? And then so I was thinking about the areas I would, I guess like it would be good to know something and more of like a dynamic sort of thing. So what happens typically when things go wrong? Sort of like what's their like, what's their base, what what are their expectations and what. But to me it's also like, how do you do that? Because typically that is an open ended question. If you if you have that, then clients might either just put in one word and you learn nothing or, you know, or they just put too much. So but typically what I'm what I'm thinking is it just wouldn't list like topics of like, where do we disagree? Where do we find the most? It would also focus on some of that like pattern or like how do we get cycled? And what else? I do like the DAS because it does sort of show like it's different, right? I like the different parts of it. And so it also the last question around how much does this actually matter? How much are you willing to put in something like that? So. Yeah, but practically speaking, something that's not too long, obviously. So it doesn't bother the client. Something that's easy to score and that gives you some cutoffs and that gives information. Yeah. I don't. I don't have anything else right now.

Interviewer: *Okay. So we are planning to carefully develop this scale and validate it over the course of two more years after this.*

Therapist: Right.

Interviewer: *And I hate to ask this question, but I'll ask it anyway, after which we will we will be selling the scale until we reduce the development cost for the development and the validation.*

Therapist: Right.

[Portion Redacted]

Interviewer: *All right I'll go with you are the you're already helping the development of scale by this interview but I also want to ask, are you interested in further collaborating for the development of the skills?*

Therapist: Yeah, if you need any, like any or anything.

Interviewer: *This would practically I think this would mean when we develop this version of the scale, then you look at it like give your....*

Therapist: Right.

Interviewer: *And we will also, once we have a final version, we might go back to therapist ...Do you give you a copy of the scale and then say like, okay, you know, just use it as you.*

Therapist: Right?

Interviewer: *Later, We'll see. How that actually works off for you, okay?*

Therapist: Sure.

Interviewer: *So actually, we are wrapping up, so I just like to go over what we have discussed in the session. First, I wanted to get a better sense of your clinical practice. So you said that you practice systemic therapy and EFT gestalt and currently emotionally focused therapy, right?*

Therapist: Yeah, [Name] Yeah, Yeah.

Interviewer: *[Name] Okay. I always get that like people also call emotions focused therapy*

Therapist: So yeah, but I did, I did individual training with Greenberg as well, but I'm like for a couple of things to small Yeah.

Interviewer: *All right. In [City, Country] you did your, your systemic therapy training in the [Country] but you you've been working at a couple of therapies for [1-5] years in the [Country]. No specific type of couples but you do get because it provides [Language] services, you do get those fairly young in a mixed culture. And usually the presenting issue is sort of the communication problems or like growing distance or infidelity that they can seem to speak to each other. We talked about your assessment procedure that you use to do it before you shifted online, before the pandemic that you used as and the relationship satisfaction scale. You like that? Yes. You like to see like a quick glance of the topics of conflict. You like the cut off, but now that it's online, you find it harder to to use these skills. So you have, I believe, not use it when you're online.*

Therapist: Yeah, correct. That's correct.

Interviewer: *And you find you find videos informative. Why did you still how do you like that? The relationship satisfaction scale if you dislike like it that much like why did you keep using right.*

Therapist: Yeah yeah right right. And I was like because I was looking at it and again, like something that I think that I sort of like grandfathered in like years back in. I think it like it does couple of things, I guess like the scale, I think it's sort of clear to look at it. So sometimes I wouldn't necessarily go to scoring. I would just like place it like hand to hand, like side to side and just see like, are we like more like very dissatisfied or very satisfied? Are we like, where are we at? So it wouldn't be just something to really easy for me in terms. It's like scoring. Not necessarily scoring, but getting a feel of where we're at. And then maybe like some of the specific topics. So maybe that. But also, like having said that, there's some questions or some fields that are covered in the essay. Also, if I was to come back to using it, I think I would just re-evaluate using this as well. So not necessarily saying that it's the best.

Interviewer: *All right. So some challenges with you for these kids that some couple skip questions. But again, also, you said that when things are important anyway, it tends to come up.*

Therapist: Yeah.

Interviewer: *So for you, these are just nice to have and really important.*

Therapist: And also some. Yeah, and I didn't say that, but I'm just thinking like sometimes it's like the couples come in and like I said, like they're like, this is our last resort. We're sort of in crisis. So we'd also feel like handing him assessment. It's just like, sort of like miss a tune. And they don't necessarily or they do, but it's just like it doesn't really feel like you want to do that. So I guess it would also depend on couples. I might have couples in the past that they just decided clinically that it's not appropriate right now. And, you know.

Interviewer: *So okay, So we've also talked about your view of relationship quality. And for you, really, it's a subjective experience. How how subjective are you satisfied they are their subjective experience or their relational well-being and how they share their vulnerabilities with each other, their pain, and a sort of mutual dependence, not just like individual coping. And for you, when you talked about what differentiates couples is high relationship quality and low relationship quality for it's sort of flexibility versus rigidity and sort of this sense that for mortgage at couples, they take things more personally, they're more reactive, they have an expectation of negativity, they snap at each other. Well, for those with higher relationship quality, there is a more flexible, greater degree of flexibility. They can escalate, they can reconnect if there's a disconnect, etc.. And for you, what's really important in a relationship really is a trust, right? Like you're not..the end.*

Therapist: That together in that as well the shared connection physical intimacy and you said when I ask you whether how you can separate the relationships or from how your clients conceptualize relationships for you. It's not really it doesn't really dramatically differ on what is a good relationship for you. It's really the more background characteristics of background family history. For example, you were talking about emotional expressiveness. [Nationality] People tend to be less emotionally expressive and this might be the cause of a difference for your clients as well. And you were again talking about that really is a way to get them to speak to each other from a place of vulnerability. Right? And when we talked about the relationship as a complex system and how you target what in this complex system to focus on, you really talked about connection, that there's going to be stages. You can talk to them about love if they're always fighting. So it's really you know, and then you how you determine when to enter couples therapy, some of them, you know, for them, they're just not fit for each other. Maybe after two sessions, after 20 sessions, you find that. But also for the successful cases, these are also collaborative that, you know, both agree with the clients that that the treatment goal is that they did what it was achieved, etc. and they use space out more of these sessions, etc.. So when we also talked about the current scale that we're developing for you, it's, you know, like any scale that gives you information would still be interesting for you. You like the cut off, you want it to be practical. So evaluation, you don't want to have score it. So it should be something on the mandate to get a good sense of

where they're at. And you also want to know the sort of dynamics, the conflict dynamics, what usually happens when they go wrong, where do they fight the most? But you're not quite sure about this because you're trying to be okay.

Interviewer: *And you know, now that I think about it, it's also like it would cover like not just the conflict or negative scale, but also like how they connect in terms of like what's good, what's so it's a strength as well. So that would be a good one. I don't think that it always is.*

Therapist: All right. How do they connect? All right. All right. And so for you, of course, you were talking about our country specific probably like market rates as well or.

Interviewer: *All right. So that would be a summary of it. Is there anything you like that you covered? All right, now, before we end, I want to talk to you about how we will handle the data. Of course, with the project, we want it to be as open as ethically possible, as verifiable, as transparent as we can for the research journal. So I wanted to know which data sharing option you are more comfortable with. One option is that transcript. Portions can be shared, none of which will be can be identified to you. So if you imagine the topics, it will be like this topic, this anonymized block of text. Another option, if you were bolder, is that the whole transcript of yours can be shared, removed of sensitive and overly identifying details. So it will be anonymous. Although in this option it will be anonymous. Will get more context. But in this option there is a minimal chance that someone who knows you professionally well can piece together the details from the transcript and narrow it down to who among their colleagues. They believe this, but again, no sensitive information will be shared, right?*

Therapist: Yeah. Well, I guess I'll go with the first, if that's okay.

Interviewer: *Sure. All right. Transcript. All right. Number one, if that's. Yeah, I think we with that, I think.*

Therapist: Okay.

